# The Medical Record: Purpose, Ownership and Legal Standing

*What the file is for, who it belongs to, and why the law reads it the way it does.*

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|---|---|
| **Module** | 01 of 12 |
| **Study time** | 100 minutes |
| **Audience** | All clinical staff, nursing, medical records staff, administrators, coders, quality teams |
| **Assessment** | Knowledge check plus a 50-question scored bank |
| **Standards basis** | NABH 6th Edition IMS and PRE chapters; IMC (Professional Conduct, Etiquette and Ethics) Regulations 2002; Consumer Protection Act 2019; DPDP Act 2023 |

Almost every serious documentation failure in an Indian hospital begins with a reasonable assumption: that the record is paperwork. It is not. It is the hospital's only account of what it did for a patient, and it is read by people who were never in the room — the next clinician, the coder, the insurer, the assessor, and sometimes a judge. This module sets out what the record is for, who owns it, which laws govern it, and why a file that looks tidy can still fail the only test that matters.

**LEARNING OUTCOMES — by the end of this module you will be able to:**

- State the six distinct purposes a medical record serves and name the reader each one serves
- Distinguish what forms part of the legal medical record from what does not
- Explain the split between ownership of the physical file and ownership of the information in it
- Identify the Indian statutes, regulations and standards that govern hospital records
- Apply the IMC three-year retention rule and the seventy-two hour supply rule correctly
- Explain how an electronic record is made admissible as evidence
- Describe how an incomplete record converts a defensible episode of care into an indefensible one
- Map the NABH standards that an assessor will test through the file

## 1.1 What a medical record actually is

A medical record is the chronological, contemporaneous account of a patient's contact with a healthcare organisation: who they were, what was found, what was decided, what was done, how they responded, and what happened next. The definition carries three words that do most of the work. **Chronological** means the sequence is preserved and visible, so a reader can see what was known at each point. **Contemporaneous** means written at or close to the time of the event, not reconstructed afterwards. **Account** means it must make sense to someone who was not present.

A file can be complete in the sense that every form is present and still fail all three tests. A progress note stamped with yesterday's date because the pad was not changed is not contemporaneous. A set of notes with entries squeezed into margins is not chronological. A note that reads "seen, continue" is not an account of anything. The discipline this course teaches is not about having more forms. It is about each entry carrying meaning to a stranger.

**KEY POINT**

The working test for every entry you write: could a competent clinician who has never met this patient pick up the file, read your entry, and know what you found, what you concluded, and what you intended to happen next? If not, the entry is incomplete, however neat it looks.

## 1.2 The six jobs the record has to do

Documentation feels like an administrative burden mainly because its purposes are invisible at the bedside. Each of the six purposes below has a different reader with different needs, and almost every argument about “how much to write” is really a disagreement about which reader is being served.

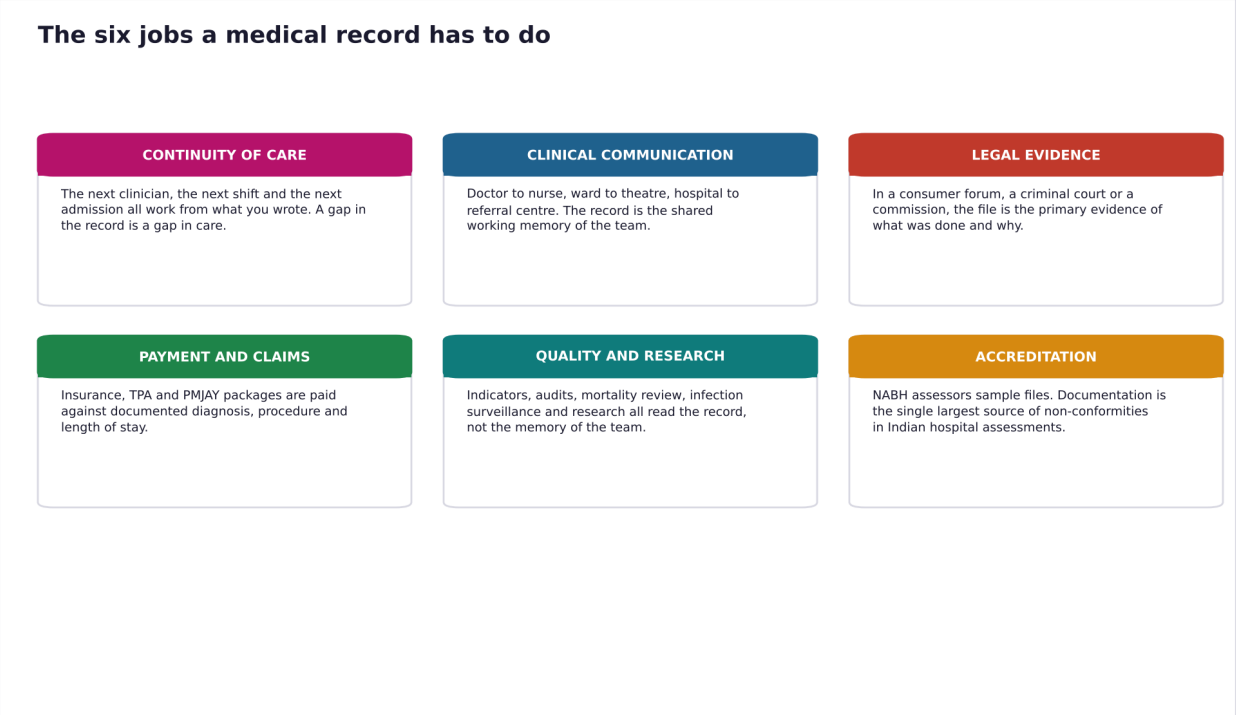



*Figure 1.1 — The six purposes of a medical record, and the reader each one serves.*

Continuity of care is the purpose clinicians feel most directly, and it is the one that fails most often at the boundaries: shift change, ward transfer, weekend cover, and the handover from a consultant who knows the patient to a registrar who does not. The other five purposes are felt later, which is exactly why they are neglected in the moment. The claim is queried six weeks after discharge. The assessor arrives eighteen months later. The notice from the consumer forum arrives three years later, by which time nobody involved remembers the patient and the file is the only witness the hospital has.

### Why the later readers matter to a clinician

It is tempting to treat the non-clinical purposes as somebody else’s problem — the billing department’s, the quality team’s, the lawyer’s. In practice they return to the clinician. An insurer rejecting a claim because the record does not justify the length of stay produces a patient complaint that lands with the treating doctor. A coder who cannot find a documented diagnosis produces a query that must be answered by the clinician who wrote the notes, months later, from memory. The time saved by writing less at the bedside is spent several times over afterwards.

## 1.3 What counts as part of the record, and what does not

The boundary matters because everything inside it is disclosable, auditable and discoverable in legal proceedings, and everything outside it is, in evidentiary terms, nothing at all. Staff frequently get this backwards: they keep informal notes they believe are private and which are in fact part of the record once filed, and they rely on information held in ways that have no evidential value at all.

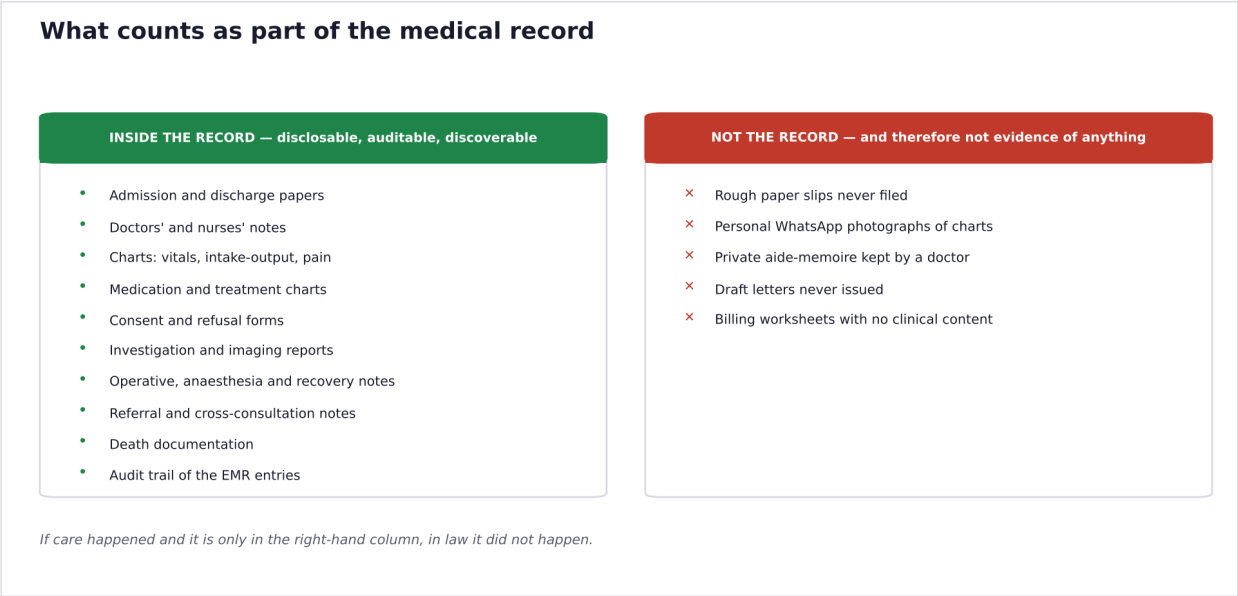



*Figure 1.2 — The boundary of the legal medical record.*

Two modern habits deserve naming. The first is photographing a chart, a wound or a report on a personal phone and sending it on a messaging app. The clinical act may be reasonable; the record of it is not, because the image sits outside the record, outside the hospital's control and inside a personal device. Under the DPDP Act 2023 it is also a transfer of personal health data outside the hospital's security arrangements. If the image informed a clinical decision, the finding belongs in the record as a written entry, and any image that forms part of the clinical account belongs in the hospital system.

The second is the private aide-memoire — the doctor's own notebook or the ward's informal handover sheet. These are not inherently improper, but they create a dangerous asymmetry: the hospital's formal record says less than the staff actually knew. When the two accounts are compared later, the hospital is in the position of arguing that its own record understates the care given, which is a position no hospital wins from.

> **CAUTION**
> Anything written about a patient that is kept, shared or relied upon has a realistic chance of being treated as part of the record when it is examined. Write every note as though it will be read aloud in a courtroom by somebody hostile to you, because the ones that matter are.

## 1.4 Who owns the file and who owns the information

This single distinction resolves most access disputes. The hospital owns the physical file, or the server on which the electronic record sits. It is the hospital's property, it stays in the hospital's custody, and the hospital is responsible for its safekeeping. The patient owns the information recorded in it. That is why a patient is entitled to a copy and is not entitled to walk out with the original; and why the hospital cannot refuse a copy because of an unpaid bill, which is a commercial dispute and not a ground for withholding health information.

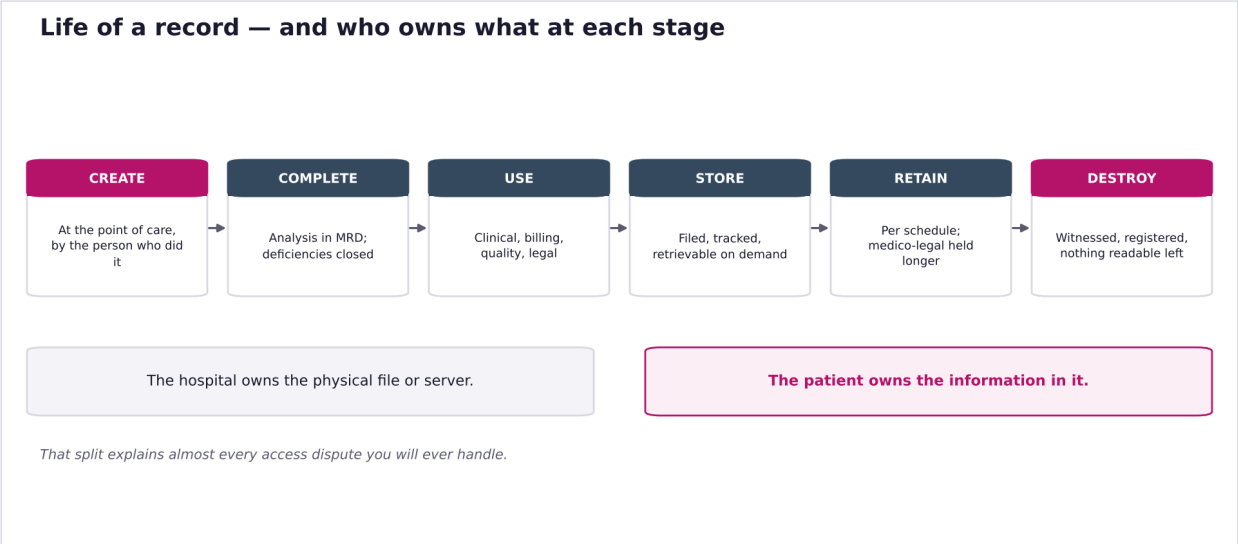



*Figure 1.3 — The life of a record, and the ownership split that governs access to it.*

The practical consequences run through every later module. Originals leave the building only for a court, against a receipt. Copies are certified so that nothing can be added afterwards. Release to anybody other than the patient needs either the patient's authorisation or a specific legal power. The custodian role — usually the medical records officer — exists precisely because the hospital holds something on behalf of someone else.

## 1.5 The legal framework in India

There is no single Indian statute called the Medical Records Act. Instead, half a dozen instruments overlap, each imposing its own duties, and a hospital must satisfy all of them. Confusion arises when staff learn one requirement — usually the three-year retention rule — and treat it as the whole law.

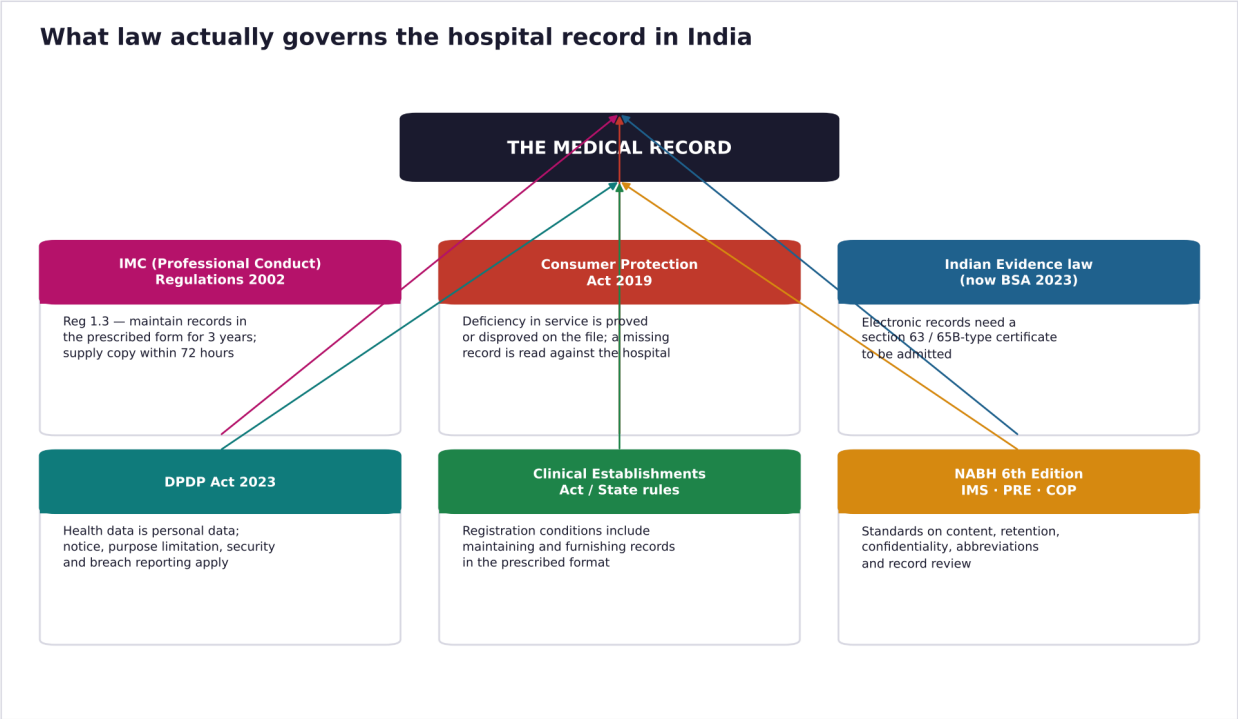



*Figure 1.4 — The instruments that together govern the hospital record in India.*

| Instrument | What it requires of records | Who enforces it |
|---|---|---|
| IMC (Professional Conduct, Etiquette and Ethics) Regulations 2002 | Maintain indoor patient records in the prescribed form for three years from commencement of treatment; supply documents to the patient or an authorised person within seventy-two hours of request | National Medical Commission and State Medical Councils, through professional misconduct proceedings against the individual doctor |
| Consumer Protection Act 2019 | No direct record-keeping clause, but deficiency in service is proved or disproved on the file; an absent or altered record is construed against the hospital | District, State and National Consumer Commissions |
| Indian evidence law (Bharatiya Sakshya Adhiniyam 2023, succeeding the Indian Evidence Act 1872) | Electronic records require a certificate from a person in a responsible position regarding the device and process that produced the output, to be admitted | Courts, at the point of admitting the record in evidence |
| Digital Personal Data Protection Act 2023 | Health data is personal data: lawful basis, notice, purpose limitation, accuracy, reasonable security safeguards, breach notification, and erasure on withdrawal subject to legal retention duties | Data Protection Board of India |
| Clinical Establishments (Registration and Regulation) Act 2010 and state equivalents | Maintain and provide records and statistics in the prescribed formats as a condition of registration | State and district registering authorities |
| NABH 6th Edition standards (IMS, PRE, COP, MOM, AAC, ROM) | Content, authentication, retention, confidentiality, abbreviation control, record review and the audit of all of these | NABH assessors, through document review and file sampling |
| Specific statutes: MTP Act, PCPNDT Act, HIV/AIDS Act 2017, Mental Healthcare Act 2017, THOTA, Narcotic Drugs rules | Prescribed forms, prescribed registers, prescribed retention periods and specific confidentiality duties for defined categories of care | The respective statutory authorities and inspectorates |

**KEY POINT**
Where two instruments differ, the hospital must satisfy the stricter one. A hospital that destroys a medico-legal file after three years because “the IMC rule says three years” has complied with nothing and exposed itself to everything.

## 1.6 The three-year rule and the seventy-two hour rule

Regulation 1.3 of the IMC Regulations 2002 is the provision most often quoted and most often misquoted. It requires that indoor patient records be maintained in the prescribed form for three years from the commencement of treatment, and that where a patient or an authorised person requests medical records, these be supplied within seventy-two hours.

Three misreadings are common. First, that three years is the correct retention period: it is a professional minimum for the individual doctor, not a ceiling, and it is displaced upward by limitation periods, medico-legal exposure, accreditation requirements and state rules. Second, that seventy-two hours is a target: it is an outer limit, and the clock runs from the request, not from when the department gets around to it. Third, that the obligation is discharged by producing some of the file: the request is for the records, and a selective release invites the inference that something is being withheld.

**CAUTION**
Refusing or delaying a patient’s request for their own records is one of the fastest ways to convert a clinical complaint into litigation. The delay itself becomes the grievance, and in proceedings it is read as consciousness of a problem in the record.

## 1.7 The record as evidence

In a consumer forum, the hospital is defending an allegation that its service was deficient. The hospital does not generally have witnesses who recall the patient; it has a file. Three practical rules follow, and they explain nearly all the documentation discipline in later modules.

1. **What is not written is treated as not done.** It is not that the court believes the care did not happen; it is that the hospital cannot prove it did, and the burden of showing that reasonable care was taken sits with the hospital once a prima facie case is made out.
2. **What is written is treated as what happened.** A contemporaneous record carries weight precisely because it was made before anyone had reason to be careful. That cuts both ways: a careless phrase written at 2 a.m. is evidence against the hospital years later.
3. **What is altered is treated as concealment.** An obliterated entry, a page replaced, a note that is demonstrably not contemporaneous — each of these damages the hospital far more than the original error would have.

### A worked example

A patient with chest pain is seen in casualty, assessed thoroughly, given an ECG that is correctly interpreted as normal, observed for four hours, reassessed, advised to return if symptoms recur, and discharged. The care is entirely reasonable. The record reads: “Chest pain. ECG NAD. Observed. Discharged. Review SOS.” No times, no reassessment, no record of what the patient was told, no copy of the ECG filed, no signature with a registration number.

The patient dies at home two days later. The family alleges negligent discharge. The hospital now has to prove that the observation happened, that reassessment was clinically adequate, that the ECG was interpreted by a competent person, and that specific return advice was given. It can prove none of these. The care was good and the defence is hopeless, which is the single most expensive sentence in hospital risk management.

## 1.8 Electronic records and admissibility

Where the record is electronic, the hospital must be able to show that the printout or export it produces is a true reflection of what the system holds. Indian evidence law requires a certificate from a person occupying a responsible official position in relation to the device or system, identifying the electronic record, describing how it was produced, and giving particulars of the device, and stating that the computer was operating properly over the relevant period.

The operational consequence is that someone in the hospital must be able and authorised to sign that certificate, and the system must be capable of supporting what the certificate asserts. A hospital whose EMR allows any user to edit a past entry without a trace cannot honestly certify the record, which means its electronic record is weaker evidence than paper. Module 12 returns to this in detail.

> **KEY POINT**
> Going paperless does not by itself strengthen the record. It strengthens the record only if entries are attributed to individual named users, timestamps are system-generated, changes are versioned rather than overwritten, and the audit trail can be produced on demand.

## 1.9 NABH 6th Edition: which standards the file is tested against

Documentation is not confined to one chapter of the standard. It is tested through almost all of them, because the assessor’s method is to pick a standard, ask how the hospital achieves it, and then open files to see whether that is what actually happens.

| Chapter | What the assessor tests through the record |
|---|---|
| AAC — Access, Assessment and Continuity of Care | Initial assessment within the defined timeframe, reassessment at defined intervals, care plans, transfer and handover documentation, discharge summary contents and timeliness |
| COP — Care of Patients | Documentation for high-risk care: resuscitation, blood transfusion, restraint, sedation, surgical and anaesthesia records, vulnerable patients, end-of-life care |
| MOM — Management of Medication | Prescription legibility and completeness, allergy documentation, verbal order policy and countersignature, administration records, adverse drug reaction reporting, medication reconciliation |
| PRE — Patient Rights and Education | Informed consent content and process, refusal documentation, information given to patients, privacy and confidentiality, access to records |
| IMS — Information Management System | The record itself: contents, standardised formats, unique identification, authentication, abbreviation control, retention, destruction, confidentiality, security, record review and the corrective actions arising |
| PSQ — Patient Safety and Quality Improvement | Documentation audit as a quality activity, indicators on completeness and timeliness, incident records, sentinel event documentation, evidence of improvement across audit cycles |
| ROM — Responsibilities of Management | The committee structure that owns records, minutes evidencing review, and the policies themselves being current, approved and version-controlled |

## 1.10 The four currencies of poor documentation

Hospitals tend to discover the cost of documentation in whichever currency hurts them first, and then treat that as the whole problem. All four are running at once.

| Currency | How the loss appears | Typical scale in a mid-size hospital |
|---|---|---|
| Clinical harm | A drug given twice because administration was not signed; an allergy missed because it was recorded on a page nobody reads; deterioration not escalated because the chart was not totalled | Low frequency, catastrophic severity; the events that become sentinel incidents |
| Revenue | Claim deductions and denials for undocumented diagnoses, unsupported length of stay, missing consent, illegible prescriptions, absent implant details | Continuous and quietly large; commonly the single biggest documentation cost |
| Accreditation | Non-conformities at assessment, deferred accreditation, repeated corrective action plans and re-assessment costs | Episodic, concentrated, and visible to senior management |
| Legal | Indefensible cases, higher settlements, professional misconduct findings against individual clinicians | Rare, very large, and always traced back to entries written years earlier |

## 1.11 Who writes what: the responsibility map

Most documentation gaps are ownership gaps. A document that everybody can write is a document nobody writes. The hospital should be able to name, for each document, the role responsible and the time by which it must exist.

| Document | Who writes it | By when |
|---|---|---|
| Initial medical assessment (history and examination) | Admitting doctor or the consultant's team | Within twenty-four hours of admission, or sooner where the hospital policy or the patient's condition requires |
| Initial nursing assessment | Nurse receiving the patient | Within the time defined in policy, commonly one to four hours of admission |

| Document | Who writes it | By when |
|---|---|---|
| Progress notes | Every clinician who sees the patient | At the time of the visit, every day of admission at minimum |
| Nursing notes and flow sheets | Nurse caring for the patient | Every shift, and at the time of each event or intervention |
| Medication orders | Prescriber only | Before administration; verbal orders countersigned within twenty-four hours |
| Consent | The clinician performing or responsible for the procedure | Before the procedure and before premedication |
| Operative note | Operating surgeon | Immediately after the procedure, before leaving the theatre suite |
| Discharge summary | Treating team, authenticated by the consultant | At discharge, with a copy handed to the patient |
| Death summary and certificate | Treating team and the certifying doctor | At the time of death and before the body is handed over |

## 1.12 The record review process

NABH requires that medical records be reviewed periodically, that the review cover both complete and in-progress records, and that it examine legibility, completeness, authentication, and whether the record tells a coherent clinical story. This is a specific, documented activity, not a general impression.

- A defined sample, drawn monthly, covering all specialties and all wards rather than whichever files are conveniently to hand
- A standard review tool with explicit criteria, so two reviewers scoring the same file reach the same answer
- Review of active files as well as discharged ones — the problems in an in-progress record are the ones that can still be fixed
- Review by people who did not write the notes, which means a multidisciplinary group rather than the department auditing itself
- Findings analysed by pattern, not by individual, and reported to the committee that can act on them
- Actions with named owners and dates, and a re-audit that uses the same tool so that improvement can be demonstrated

## 1.13 Myths that cause real damage

| What staff often believe | What is actually the case |
|---|---|
| “If I write less, there is less to be used against me.” | The opposite. Thin notes remove the hospital’s ability to show that reasonable care was taken. Detailed, honest notes are the defence. |
| “Nursing notes are not really part of the legal record.” | They are, and in disputes about timing, deterioration and what was reported to whom, they are often the most important part of it. |
| “The patient can’t get the records until the bill is cleared.” | The information belongs to the patient. A billing dispute is a separate matter and is not a lawful ground for withholding records. |
| “We can correct it later when there is time.” | A correction made later is a late entry and must be labelled as one. Correcting silently afterwards is the behaviour that destroys credibility. |
| “It is in the system, so it is documented.” | Only if the entry is attributed, timed and retrievable, and only if what the clinician actually assessed is what the template recorded. |

| What staff often believe | What is actually the case |
|---|---|
| “We keep everything forever, so retention is not our problem.” | Indefinite retention without a policy breaches data-minimisation duties and produces files that cannot be retrieved when they are needed. |

## 1.14 What good looks like

A well-documented admission is recognisable within thirty seconds of opening the file. Every page carries the patient’s identifiers. The admission note states why the patient came, what was found, what was considered and what was planned. Each progress note moves the story forward and is signed, timed and attributed. The nursing chart and the medical notes describe the same patient. Consent forms sit with the procedures they authorise. Investigation results that mattered are mentioned in the notes with the action taken. The discharge summary could be handed to a doctor in another city and they would know what to do next.

None of that requires more time than poor documentation. It requires the entries to be written once, properly, at the time — rather than written thinly at the time and reconstructed expensively afterwards.

## 1.15 Reading a file the way an assessor reads it

An assessor does not read a file from cover to cover. They sample, and they sample against a standard they have already chosen. Understanding the method is useful, because the same method is the fastest way for a hospital to audit itself.

1. **Pick the standard first.** For example, the requirement that an initial assessment is documented within a defined timeframe.
2. **Ask what the hospital says it does.** Read the policy, note the timeframe the hospital has set for itself, and note who it says is responsible.
3. **Open files to see whether that is what happens.** Not a convenient file — a weekend admission, a transfer from casualty, a patient admitted at night.
4. **Follow one thread across documents.** If the medical note says a drug was started, check the chart; if the chart shows a transfusion, check the consent and the observations; if the nursing chart shows a fall, check the incident record and the medical note.
5. **Test the exceptions.** The DAMA file, the death file, the medico-legal file, the restraint episode, the patient who absconded. Exceptions are where systems fail.
6. **Close with the committee minutes.** Do they show the hospital finding and fixing these things itself?

A hospital that runs this exercise on itself once a month, with a clinician and a records officer together, will find almost everything an assessor would find, and will find it while there is still time to fix it.

## 1.16 The record across different care settings

Everything in this course applies across settings, but the shape of the risk changes with the setting, and hospitals usually protect the inpatient file best because it is the most visible.

| Setting | Dominant documentation risk |
|---|---|
| Outpatient clinic | High volume and short contacts, so examination findings and advice go unrecorded; the record cannot later show what the patient was told |
| Emergency department | Times missing, reassessment absent, medico-legal screening overlooked, and disposal decisions recorded without a reason |
| Intensive care | Volume of data obscures the narrative; flow sheets are complete while the reasoning behind changes in therapy is nowhere |
| Operating theatre | Nine documents with different owners; the ones without a named owner go missing |

| Setting | Dominant documentation risk |
|---|---|
| Dialysis and day care | Repetitive sessions documented by template, so a change in the patient's condition is not visible against the background |
| Obstetrics | Partogram and fetal monitoring documentation are decisive in the highest-value claims in Indian hospitals |
| Psychiatry | Capacity, consent, restraint and confidentiality all carry statutory overlays that the general record set does not handle |
| Home care and teleconsultation | The encounter leaves no hospital-side trace unless it is deliberately created; telemedicine guidelines require the consultation to be recorded as any other |

## 1.17 What the patient sees

Patients increasingly read their own records, and the DPDP Act and accreditation standards both point in the direction of greater access. This changes what it is wise to write, not by making notes less honest, but by making them more careful.

- Write about the patient as though they will read it, because they may
- Avoid shorthand that reads as dismissive: 'poor historian', 'pleasant obese female', 'denies' where 'reports no' is accurate
- Record what the patient says they are worried about; it is clinically useful and it demonstrates that they were heard
- Where a conversation is difficult, record its content rather than your view of the patient's character
- Explain abbreviations in the discharge summary, which is the document the patient will certainly read

> **KEY POINT**
> Nothing in a record should be written that could not be said to the patient's face. That single rule removes almost every entry that later causes a hospital difficulty.

## 1.18 Putting this module to work

1. Open the last file you wrote in and read your own most recent entry as a stranger would. Can a reader tell what you found, what you concluded and what you planned?
2. Check that every page of that file carries the patient's name and record number.
3. Find out, today, who in your hospital is the custodian of medical records and how a patient actually requests a copy.
4. Find out what your hospital's stated retention period is, and whether medico-legal files are flagged so they are not destroyed on the ordinary schedule.
5. If your hospital uses an EMR, find out whether a past entry can be edited without leaving a trace, and who can sign the certificate that makes an electronic record admissible.

# Knowledge Check

Work through each item before reading the model answer. These are applied questions: the answer is the standard of documentation your assessor, your medico-legal adviser or the next clinician to read the file would expect.

Q1. **A patient's relative demands the original file at discharge, saying the family paid for the treatment so the records are theirs. How do you answer, and what do you do?**

**Answer:** Explain the ownership split: the hospital owns the physical file and is legally responsible for its custody; the patient owns the information. The family is therefore entitled to a copy, not the original. Take a written request, verify identity and the relationship, obtain the patient's authorisation if the patient is alive and has capacity, and supply certified copies within seventy-two hours. Record the release in the release register. Do not make the release conditional on the bill being settled.

Q2. **A consultant tells you that the notes should be kept brief because "the less that is written, the less a lawyer can attack". What is wrong with this reasoning?**

**Answer:** It inverts the way evidence works. Once a prima facie case of deficiency is raised, the hospital must show that reasonable care was taken, and the record is how it does that. Thin notes do not remove material from the plaintiff; they remove the defence. The correct advice is to write accurately, objectively and completely, and to avoid speculation, blame and informal commentary — not to write less.

Q3. **Your hospital destroyed an inpatient file after three years, citing the IMC regulation. A summons now arrives relating to that admission, which had been registered as a medico-legal case. What has gone wrong?**

**Answer:** Three years is a professional minimum for the individual doctor, not a hospital retention policy, and it never applies to a medico-legal file. Medico-legal records must be flagged at creation so they are excluded from routine destruction, and must be retained while any proceeding is possible — in practice ten years or longer. The failure is a process failure: the destruction list was not screened against the MLC register.

Q4. **The hospital has moved to an EMR in which a doctor can edit a progress note written three days ago, with the note simply showing the new text. Why is this a problem beyond mere tidiness?**

**Answer:** It destroys both contemporaneity and admissibility. A reader cannot tell what was recorded at the time of care, so the record no longer evidences what was known when decisions were made. It also makes it impossible to honestly certify that the output is a true reflection of entries made at the relevant time, which is what Indian evidence law requires for an electronic record. Edits must be versioned: the original preserved, the change recorded with author, timestamp and reason.

Q5. **A nurse photographs a wound on her own phone and sends it to the consultant on a messaging app, and the consultant changes the dressing plan on that basis. What should now happen?**

**Answer:** The clinical decision must be documented in the record: a written entry describing the wound appearance, who was consulted, what was advised, and the time. If an image forms part of the clinical account it belongs in the hospital system under the hospital's clinical photography consent and security arrangements, not on a personal device. The personal copy should be deleted. As it stands, the hospital holds a decision with no documented basis, and personal health data has left its control, which also engages the DPDP Act.

Q6. **An assessor opens a file at random and finds a well-written medical record with no nursing notes for a thirty-six hour period over a weekend. What will they conclude, and against which standard?**

**Answer:** They will conclude that care was either not delivered or not documented, and that the hospital cannot distinguish between the two. It is tested against the AAC reassessment requirements and the IMS requirements on record contents, and it also undermines COP standards for any high-risk care given during that period. The finding is particularly damaging because a gap over a weekend suggests a systemic staffing or supervision issue rather than one clinician's lapse.

**ASSESSMENT BANK**

A fuller bank of 50 scored questions for this module is supplied separately as a JSON question bank for loading into your LMS.